

ADMISSION NOTE - Patient synth_03 - Community-acquired pneumonia

PATIENT: Pendelton, Arthur

MRN: synth_03

DATE OF ADMISSION: 2023-10-21

CHIEF COMPLAINT: Shortness of breath and cough x 3 days.

HISTORY OF PRESENT ILLNESS: Mr. Pendelton is a 68-year-old male with a past medical history of HTN, hyperlipidemia, and GERD who presents to the ED with a 3-day history of worsening productive cough, fevers, and shortness of breath. He reports the cough produces thick, yellow-green sputum and is associated with right-sided pleuritic chest pain. He has been checking his temperature at home, which has been up to 39.0 C. He feels generally weak and fatigued. He denies any recent travel, known sick contacts, or hospitalization. He tried over-the-counter dextromethorphan with minimal relief.

PAST MEDICAL HISTORY:

1. Hypertension
2. Hyperlipidemia
3. Gastroesophageal reflux disease (GERD)

SOCIAL HISTORY: Retired librarian. Lives at home with his wife. Quit smoking over 20 years ago (~15 pack-year history). Drinks 1-2 glasses of wine per week. Denies illicit drug use.

REVIEW OF SYSTEMS: As per HPI. Positive for fever, chills, productive cough, dyspnea, and pleuritic chest pain. Negative for nausea, vomiting, diarrhea, abdominal pain, headache, or focal weakness.

PHYSICAL EXAM:

VITALS: T 38.9 C, HR 112 bpm, BP 145/90 mmHg, RR 24, SpO2 91% on room air.

GENERAL: Appears uncomfortable and in mild respiratory distress.

HEENT: Mucous membranes are moist.

LUNGS: Crackles and decreased breath sounds appreciated at the right lung base. No wheezing.

CARDIOVASCULAR: Tachycardic but regular rhythm. No murmurs, rubs, or gallops.

ABDOMEN: Soft, non-tender, non-distended. Bowel sounds are normal.

EXTREMITIES: No cyanosis, clubbing, or edema.

IMAGING: Chest X-ray shows a right lower lobe consolidation consistent with pneumonia.

ASSESSMENT AND PLAN:

1. ****Community-Acquired Pneumonia (CAP):**** Hypoxic pneumonia requiring admission. CURB-65 score is 1 (age >65), suggesting inpatient treatment. Will admit to General Medicine service.
 - Start IV Ceftriaxone 1g daily and IV Azithromycin 500mg daily.
 - Provide supplemental O2 via nasal cannula, titrate to keep SpO2 >92%.
 - Obtain blood and sputum cultures.
 - Order urinary antigen tests for Strep pneumo and Legionella.
2. ****Hypertension:**** Continue home medication (Lisinopril). Monitor blood pressure.
3. ****Hyperlipidemia:**** Continue home medication (Atorvastatin).
4. ****GERD:**** Continue home medication (Pantoprazole).
5. ****Rule out Hyponatremia:**** Monitor electrolytes, as SIADH can be a complication of pneumonia.
6. ****VTE Prophylaxis:**** Start subcutaneous Heparin.

PROGRESS NOTE ? Day 1 - Patient synth_03 - Community-acquired pneumonia

PATIENT: Pendelton, Arthur

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DATE: 2023-10-22

SUBJECTIVE: Patient reports feeling somewhat better this morning. His shortness of breath has improved, though he still feels winded when getting out of bed. The right-sided chest pain is now a dull ache. Cough is less frequent but still productive of yellow sputum. No new complaints.

OBJECTIVE:

VITALS: T 38.1 C, HR 95, BP 138/85, RR 20, SpO2 94% on 2L NC.

EXAM: Lungs reveal persistent crackles at the right base, though they seem less coarse than on admission. Cardiac exam shows regular rate and rhythm.

LABS: WBC is down to 13.5 from 15.2. Basic metabolic panel is stable. Blood cultures collected on admission show no growth after 24 hours. Sputum culture is pending.

ASSESSMENT AND PLAN:

1. ****Community-Acquired Pneumonia:**** Showing early signs of response to therapy. Fever is lower and oxygen requirement has decreased slightly.
 - Continue IV Ceftriaxone and Azithromycin.
 - Continue to wean O2 as tolerated with goal of maintaining SpO2 >92%.
 - Encourage incentive spirometry.
2. ****Hypertension:**** Blood pressure is well-controlled on home medication.
3. ****Hyperlipidemia:**** Continue Atorvastatin.
4. ****VTE Prophylaxis:**** Continue Heparin SQ.

Plan for today is to continue current management. If he becomes afebrile for 24 hours and is weaned off oxygen, we can consider transitioning to an oral antibiotic regimen for discharge.

PROGRESS NOTE ? Day 2 - Patient synth_03 - Community-acquired pneumonia

PATIENT: Pendelton, Arthur

MRN: synth_03

DATE: 2023-10-23

SUBJECTIVE: Patient reports significant improvement. He was afebrile overnight. He states his breathing is 'almost back to normal' and he has no chest pain. He ambulated in the hallway with physical therapy without desaturation. He is tolerating a regular diet.

OBJECTIVE:

VITALS: T 37.1 C, HR 84, BP 130/80, RR 18, SpO2 96% on room air.

EXAM: In no apparent distress. Lungs are clear to auscultation except for some very faint crackles at the right base which have improved significantly. He is speaking in full sentences comfortably.

LABS: WBC is 9.8. Blood cultures are final with no growth. Sputum culture shows only normal respiratory flora. Strep pneumo urinary antigen is negative.

ASSESSMENT AND PLAN:

1. **Community-Acquired Pneumonia:** Clinically resolved. He meets criteria for discharge: afebrile, hemodynamically stable, and no longer requiring supplemental oxygen.
 - Discontinue IV antibiotics.
 - Prescribe course of oral antibiotics to complete treatment: Amoxicillin-clavulanate 875/125mg BID for 5 days and Azithromycin 500mg daily for 2 days.
2. **Disposition:** Patient is stable for discharge home. He lives with his wife and has a safe home environment. PT has cleared him. Case management has arranged for follow-up with PCP. Prescriptions sent to pharmacy. Discharge paperwork and education provided.

LABORATORY REPORT - Patient synth_03 - Community-acquired pneumonia

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--- COMPLETE BLOOD COUNT ---

Lab | 10/21 (Adm) | 10/22 | 10/23 (D/C)

WBC (K/uL) | 15.2 (H) | 13.5 (H) | 9.8

Hgb (g/dL) | 14.1 | 14.0 | 13.9

Hct (%) | 42.3 | 42.1 | 41.8

Plt (K/uL) | 255 | 240 | 235

--- COMPREHENSIVE METABOLIC PANEL (10/21) ---

Sodium (mEq/L) | 138 | Range: 136-145

Potassium(mEq/L)| 4.1 | Range: 3.5-5.1

Chloride (mEq/L)| 102 | Range: 98-107

CO2 (mEq/L) | 24 | Range: 22-29

BUN (mg/dL) | 22 (H) | Range: 7-20

Creatinine(mg/dL)| 1.1 | Range: 0.7-1.3

Glucose (mg/dL) | 105 | Range: 70-110

AST (U/L) | 25 | Range: 10-40

ALT (U/L) | 28 | Range: 7-56

--- MICROBIOLOGY ---

Blood Culture x2 (10/21): No Growth at 48 hours.

Sputum Culture (10/21): Normal respiratory flora.

Strep Pneumo Urinary Ag (10/21): Negative.

Legionella Urinary Ag (10/21): PENDING.

MEDICATION RECORD - Patient synth_03 - Community-acquired pneumonia

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--- ADMISSION MEDICATIONS (Active during stay) ---

1. Ceftriaxone 1 gram IV, ONCE A DAY
2. Azithromycin 500 mg IV, ONCE A DAY
3. Lisinopril 20 mg PO, ONCE A DAY
4. Atorvastatin 40 mg PO, ONCE A DAY
5. Pantoprazole 40 mg PO, ONCE A DAY
6. Heparin 5000 units SUBCUT, THREE TIMES A DAY (for VTE prophylaxis)
7. Acetaminophen 650 mg PO, EVERY 6 HOURS AS NEEDED for fever/pain

--- DISCHARGE MEDICATIONS (Prescribed at discharge 10/23) ---

1. Amoxicillin-Clavulanate 875-125 mg Tablet
 - SIG: Take 1 tablet by mouth two times a day for 5 days.
2. Azithromycin 250 mg Tablet
 - SIG: Take 2 tablets (500mg total) by mouth once a day for 2 days.
3. Lisinopril 20 mg Tablet
 - SIG: Continue taking 1 tablet by mouth once a day.
4. Rosuvastatin 20 mg Tablet
 - SIG: Take 1 tablet by mouth once a day.
5. Pantoprazole 40 mg Tablet
 - SIG: Continue taking 1 tablet by mouth once a day.
6. Acetaminophen 650 mg Tablet
 - SIG: Take 1 tablet by mouth every 6 hours as needed for pain.

DISCHARGE ADVICE - Patient synth_03 - Community-acquired pneumonia

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DISCHARGE DATE: 2023-10-23

DISCHARGE DIAGNOSES:

1. Community-Acquired Pneumonia, resolved
2. Hypertension, chronic
3. Hyperlipidemia
4. Hyponatremia, resolved
5. Gastroesophageal Reflux Disease (GERD)

CONDITION AT DISCHARGE: Stable. The patient is afebrile, with stable vital signs. He is ambulating without assistance and his oxygen saturation is 96% on room air. He is tolerating a diet and is in good spirits.

DISCHARGE MEDICATIONS: A list of your new and continued medications has been provided separately and sent to your pharmacy. Please review the instructions carefully and take them as directed. Do not stop any medication without talking to your doctor.

ACTIVITY: You may feel tired for a few weeks. Rest when you feel tired, but it is important to get up and walk around several times a day to help your lungs recover and prevent blood clots. Avoid strenuous activity and heavy lifting for at least one week.

FOLLOW-UP: Please schedule a follow-up appointment with your primary care provider, Dr. Evans, within the next 7-10 days to ensure you are recovering well.

PENDING RESULTS: The result for the Legionella urinary antigen test is not yet back. Your primary care provider will be notified of this result when it becomes available. Please contact their office to discuss the result if you have not heard from them within 5 business days.